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``html
<!DOCTYPE html>
<html lang="en">
<head>
<meta charset="UTF-8">
<meta name="viewport" content="width=device-width, initial-scale=1.0">
<title>Synthetic Medical Record - EGD Chart</title>
<style>
body {
font-family: Arial, Helvetica, sans-serif;
margin: 0;
padding: 0;
}
.page {
page-break-after: always;
padding: 20px;
}
header, footer {
text-align: center;
font-size: 12px;
}
footer {
margin-top: 50px;
}
table {
border-collapse: collapse;
margin-bottom: 20px;
}
th, td {
text-align: left;
vertical-align: top;
padding: 5px;
border: 1px solid #000;
}
.table-fit {
width: fit-content;
max-width: 100%;
display: inline-table;
table-layout: auto;
}
.table-wide {
width: 100%;
table-layout: fixed;
}
</style>
</head>
<body>

<!-- Page 1 -->
<section class="page">
<header>
Gastro Clinic | Endoscopy Unit<br>
123 Health Ave, Wellness City, HC 54321 | Phone: 555-1234 | Fax: 555-5678<br>
Endoscopist: Dr. Jane Doe, MD
</header>

<h2>Registration / Demographics</h2>
<table class="table-wide">
<tr>
<th>Patient Name</th>
<td>John Doe</td>
<th>DOB</th>
<td>01/15/1980</td>
<th>Age</th>
<td>43</td>
</tr>
<tr>
<th>Sex</th>
```

<td>Male</td>	
<th>Address</th>	
<td>456 Main St, Sampletown, ST 12345</td>	
<th>Phone</th>	
<td>555-6789</td>	
</tr>	
<tr>	
<th>MRN</th>	
<td>MRN123456</td>	
<th>Account</th>	
<td>ACCT789012</td>	
<th>Insurance</th>	
<td>TestHealth Plan</td>	
</tr>	
<tr>	
<th>Emergency Contact</th>	
<td>Jane Doe (Spouse) - 555-9876</td>	
<th>Encounter Date</th>	
<td>07/22/2023</td>	
<th>Check-In Time</th>	
<td>08:30 AM</td>	
</tr>	
<tr>	
<th>Location</th>	
<td>Endoscopy Suite 1</td>	
<th>Attending Gastroenterologist</th>	
<td>Dr. Jane Doe, MD</td>	
<th>Referring Provider</th>	
<td>Dr. Smith, MD</td>	
</tr>	
<tr>	
<th>Chief Complaint / Indication</th>	
<td>Dyspepsia</td>	
</tr>	
<tr>	
<th>PMH</th>	
<td>GERD, Hypertension</td>	
<th>PSH</th>	
<td>Appendectomy</td>	
<th>FHx</th>	
<td>Colon cancer (father)</td>	
</tr>	
<tr>	
<th>SHx</th>	
<td>Non-smoker, Social alcohol use</td>	
<th>Allergies</th>	
<td>Penicillin</td>	
<th>Problem List</th>	
<td>Dyspepsia, GERD</td>	
</tr>	
<tr>	
<th>Current Medications</th>	
<td colspan="2">	
Omeprazole 20 mg daily 	
Lisinopril 10 mg daily 	
Metoprolol 50 mg BID 	
Aspirin 81 mg daily 	
Atorvastatin 40 mg nightly	
</td>	
</tr>	
<tr>	
<th>Pre-procedure Vitals</th>	
<td colspan="2">	
BP: 135/85 mmHg, HR: 72 bpm, SpO2: 97% on RA, ASA II	
</td>	
</tr>	
<tr>	

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<th colspan="6">
Informed consent obtained and documented in medical record.
</th>
</tr>
</table>

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<h2>Pre-Procedure Assessment & Anesthesia Record</h2>
<table class="table-fit">
<tr>
<th>Pre-Anesthesia Evaluation</th>
<td>Airway: Mallampati II, Cardio: Regular rhythm, clear lungs, NPO since midnight.</td>
</tr>
<tr>
<th>Planned Sedation</th>
<td>MAC with Propofol, Fentanyl 100 mcg, Midazolam 2 mg.</td>
</tr>
<tr>
<th>Anesthesia Team</th>
<td>Anesthesiologist: Dr. Ana Save, MD<br>CRNA: John Nurse, CRNA</td>
</tr>
<tr>
<th>Times</th>
<td>
Anesthesia Start: 09:00 AM<br>
Procedure Start: 09:15 AM<br>
Procedure End: 09:45 AM<br>
Anesthesia End: 10:00 AM<br>
Recovery Start: 10:05 AM<br>
Recovery End: 11:00 AM
</td>
</tr>
<tr>
<th>Monitoring Trends</th>
<td>BP: 130/80, 125/75, 120/70; HR: 70, 68, 72; SpO2: 97%, 98%, 99%</td>
</tr>
<tr>
<th>Complications</th>
<td>Brief desaturation to 92%, resolved with jaw thrust and O2 adjustment.</td>
</tr>
<tr>
<th>Post-Anesthesia Evaluation</th>
<td>Aldrete Score: 9/10, Disposition: Home with escort.</td>
</tr>
</table>

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<h2>Procedure Note: Esophagogastroduodenoscopy (EGD)</h2>

<p>
An EGD was performed using a standard endoscope, Model GE-Scope FX, with the patient lying in a left lateral decubitus position and a bite block in place. The endoscope was gently passed through the oropharynx without difficulty.
</p>

<p>
The esophagus was observed with the following landmarks: Z-line visualized at 40 cm from the incisors, with no rings or strictures noted. Mild esophagitis was noted with linear ulcerations 5 mm in diameter.
</p>

<p>
Inspection of the stomach revealed the body and antrum with mild erythema, with normal-appearing rugal folds. Upon retroflexion, no abnormalities in the cardia and fundus. The pylorus appeared patent and functional.
</p>

<p>
The duodenum was examined up to the second portion. The villi appeared normal with no visible ulcerations.
</p>

<h3>Findings:</h3>

- Mild esophagitis with linear ulcerations.
Normal stomach and duodenal mucosa.

<h3>Impression:</h3>

<p>Findings consistent with GERD exacerbation.</p>

<h3>Diagnosis:</h3>

- GERD with esophagitis.

<h3>Interventions:</h3>

<p>No interventions performed, biopsied esophagus (3 specimens).</p>

<p>
Team: Endoscopist - Dr. Jane Doe, MD; Assistant - Jane Smith, RN; Circulating RN - Mike Nurse, RN; Endoscopy Tech - Helen EndoTech.
</p>

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<!-- Page 4 -->

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</header>

<h2>Specimens / Lab / Pathology</h2>

<table class="table-fit">

<tr>

<th>Container</th>

<th>Site</th>

<th>Number of Fragments</th>

<th>Method</th>

<th>Fixative</th>

</tr>

<tr>
<td>A</td>
<td>Esophagus</td>
<td>3</td>
<td>Cold forceps</td>
<td>Formalin</td>
</tr>
</table>
<p>Collection Time: 09:45 AM | Sent to Lab: 10:00 AM</p>
<h3>Lab: Sample Pathology</h3>
<p>Accession Number: LAB-TST123456</p>
<h3>Pathology Report:</h3>
<p>
Gross Description: Three tan tissue fragments, 2-3 mm each.
</p>
<p>
Microscopic Description: Chronic inflammation with basal cell hyperplasia, no dysplasia.
</p>
<p>
Final Diagnosis: Chronic esophagitis consistent with GERD. Negative for neoplasia.
</p>
<p>Pathologist: Dr. Path Finder, MD | Report Date: 07/22/2023 1:00 PM</p>
<footer>
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<h2>After Visit Summary / Discharge Instructions</h2>
<h3>Recovery Course:</h3>
<p>
The patient tolerated the procedure well. No pain or nausea reported in recovery. Vitals remained stable.
</p>
<h3>Diet/Activity Instructions:</h3>
<p>Resume regular diet as tolerated. Avoid alcohol and NSAIDs for 24 hours.</p>
<h3>Warning Signs:</h3>
<p>Contact us if experiencing severe pain, persistent bleeding, or breathing difficulties.</p>
<h3>Follow-up Plan:</h3>
<p>Follow-up with Dr. Jane Doe in 2 weeks. Results discussed; call if questions arise.</p>
<h3>Medication Plan:</h3>
<p>Continue PPI therapy, adjust if symptoms persist.</p>
<h3>Patient Understanding and Escort:</h3>
<p>The above instructions were reviewed with the patient, who voiced understanding. Patient discharged with escort.</p>
<p>Discharge Educator: Lisa Teach, RN | Timestamp: 11:10 AM</p>
<p>
Electronically signed by Dr. Jane Doe, MD | 07/22/2023 11:15 AM
</p>
<footer>
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